

Daily Update — Dad's care, 2026-05-19

AT A GLANCE

- **First IV ketamine session at Royal Oak yesterday (5/18).** Difficult in the chair — shaking, crying out. Afterward Dad was more coherent than Mom has seen him in a long time; he himself said he didn't feel better.
- **Dad and Julane will continue weekly sessions through the gap before the June 8 ketamine restart.** This is a meaningful new piece of continuity — integration and behavioral work happening alongside any medication decisions rather than being held until the next infusion.
- **Royal Oak: off the table. CA: off the table per Dad.**
- **Ann Arbor clinic start: June 8** (revised from tentatively early June). Chilton pushed it back to allow the Cymbalta → Lexapro switch to complete first.
- **Four OpenEvidence consultations have come back since this morning** — on SSRI vs SNRI as ketamine background (#10), divalproex in this profile (#11), RBD episode-frequency trajectory (#12), lemborexant onset speed (#13), and bupropion augmentation in this activation-intolerance profile (#14). They land directionally. Summarized below.
- **Headline reads:** (a) the published evidence on the Cymbalta → Lexapro switch is mixed — the OE response lands against it on Del Casale 2025 + escitalopram-already-failed grounds, but Dad's decades-long prior-responder history on Lexapro is real prior-responder data the OE didn't have; the load-bearing question is now **sequencing** rather than direction (do the cross-titration *after* the first 2–3 ketamine sessions, not before); (b) divalproex has no defensible indication and serious drug-induced parkinsonism risk in Dad's profile; (c) the decreased RBD episode frequency does *not* relax the conservative medication posture and may reflect confounders (duloxetine REM suppression, melatonin, separate sleeping); (d) lemborexant is night-1 effective and a seamless substitute for doxepin if doxepin needs to come off; (e) bupropion augmentation is a defensible alternative to divalproex if augmentation beyond ketamine is desired, with fall risk as the load-bearing caveat.
- **Dad's emotional state:** dejected; explicitly asked the family to maintain genuine (not false) hopefulness.

WHAT WE KNOW

- **5/18 session at Royal Oak:** shaking, crying out, prolonged distress. "Going to Africa." No provider in the room.
- **Post-session functional state on 5/18:** "more coherent than Mom has seen him in ages." Dad said he didn't feel better in the chair. The session experience and the response are separate variables.
- **Dad's processed experience after Julane (5/19):** didn't feel attached to anyone; runaway train; not in a position to walk through the door ketamine was opening. February felt supported (family around him); 5/18 felt alone.
- **Julane continuity:** Dad will keep meeting with Julane weekly through the gap to June 8. This is the strongest practical lever on the "felt alone" / set-and-setting dimension that distinguished 5/18 from February.
- **February 2026 ketamine course was IV** and produced a dramatic ~4-week response on a duloxetine + brexpiprazole background. Dad is specifically an IV responder. Later lozenge use was good but less transformative.
- **Dad did not take the proposed quetiapine.** See [handout #18](#).
- **A2 ketamine start: June 8.** Chilton's stated rationale: allow Cymbalta → Lexapro switch to complete first.
- **Current sleep stack:** doxepin 6 mg HS, trazodone 50 mg PRN HS, melatonin 3 mg HS. See [handout #13](#) and [handout #19](#).
- **Current antidepressant:** duloxetine 60 mg, on board since 11/2025 (~6.5 months). The documented Feb 2026 responder background.

- **Bupropion has never been on board.** Considered and held in clinical brief; family lit review #11 (4/20) explicitly excluded it on adrenergic-intolerance grounds — an exclusion the OE bupropion response (see below) explicitly judged "too broad."
- **RBD episode frequency trajectory:** decreased over the past year; most recent ~February 2026; now roughly every few months; no dream recall recently. Multiple confounders make this uninterpretable as a stabilization signal (see OE summary below).
- **Sleep pattern detail:** one nocturia trip per night, no dream recall, wakes unrested, cannot nap.
- **Caffeine intake actual:** one cup with breakfast, nothing after. (Mom's report was accurate.)
- **Cognitive in-call observation (5/19):** Dad's memory was spotty and hesitant — didn't initially recall a call from ~1 hour earlier. Less impaired than San Francisco period.

OPENEVIDENCE RESPONSES — WHAT THEY TELL US

- **OE #10 — SSRI vs SNRI as the antidepressant background for IV ketamine.** Del Casale 2025 (JAMA Psychiatry, n=55,480) found SNRI + esketamine outperformed SSRI + esketamine on mortality (5.3% vs 9.1%), hospitalization, and relapse. Mechanism favors SNRI synergy with ketamine (NE neuron firing is part of ketamine's mechanism). Duloxetine discontinuation in a 3-week cross-titration leaves withdrawal-related anxiety in the same window as the ketamine restart. Escitalopram start carries activation risk that Dad's history is a documented predisposing factor for. Escitalopram was already failed at high doses in 10/2025 — though Dad has decades of strong functioning on lower doses (10–20 mg) that the OE response did not have access to. **The OE response lands against the switch in general; the family read is that the switch direction is defensible given Dad's long-term prior-responder status, but the timing relative to the June 8 ketamine restart is the real concern.** See [handout #06](#) (revised).
- **OE #11 — Divalproex in late-life depression with prodromal-synucleinopathy features.** No RCT evidence for unipolar TRD augmentation; not in any major guideline for this indication. Valproate-associated parkinsonism is well-documented, insidious in onset (months to years), dose-independent, can *unmask subclinical PD* in susceptible patients. UK Biobank data show persistent valproate-PD association after adjusting for epilepsy and excluding scripts within 5 years of PD diagnosis. Limited evidence (n=36) suggests it may attenuate ketamine response. **Ranked at or near the bottom of the augmentation hierarchy for Dad's profile.** Significant drug interaction with doxepin (2.2× higher level). See [handout #01](#) (revised).
- **OE #12 — RBD episode frequency trajectory.** Episode frequency has never been validated as a phenoconversion predictor in any cohort study. Subjective frequency can decrease while objective RSWA increases. Dad's decreased frequency has multiple confounders: duloxetine REM suppression, melatonin treatment, separate sleeping, decreased dream recall (which in DLB literature can itself be a progression marker). Dad's profile (age 76, 8.5y RBD, constipation, gait/balance findings) places him in the highest-risk Arnaldi 2021 phenoconversion stratum (HR 5.71 with age >70 + constipation + putaminal dopaminergic dysfunction; two of three criteria already met clinically). **The conservative medication posture should not be relaxed** based on the trajectory; the RBD-with-rising-RSWA dissociation in PD cohorts cuts the other direction.
- **OE #13 — Lemborexant onset speed.** Effective from night 1 — WASO reduction of 33 min (5 mg) and 42 min (10 mg) vs placebo on nights 1–2 in adults ≥55. Second-half-of-night WASO benefit present from night 1. Switching from doxepin 6 mg to lemborexant 5 mg can be done seamlessly — both produce first-night WASO effects, no gap expected. No rebound insomnia, no withdrawal; can be stopped abruptly. CYP3A4-metabolized; no significant PK interaction with duloxetine or low-dose doxepin. **Low-risk trial.** See [handout #10](#) (revised).
- **OE #14 — Bupropion augmentation in this activation-intolerance profile.** Family lit review #11's exclusion of bupropion was judged "too broad" — bupropion has no meaningful serotonergic activity, and mirtazapine α2 antagonism (which produced Dad's agitation) is mechanistically distinct from bupropion NRI. OPTIMUM (Lenze 2023): bupropion augmentation = 28.2% remission, comparable to aripiprazole augmentation (28.9%) and superior to bupropion switch (19.3%). **Bupropion as augmentation is defensible; bupropion as a switch is the worst-performing OPTIMUM arm** — relevant if "switch antidepressant" stays on the table. Cardiac profile favorable for Dad's bradycardia + LAFB. Bupropion + ketamine compatible (Cook & Halaris 2022 proposes NDRI enhancement may improve ketamine response durability). Load-bearing caveat: **highest fall rate of any OPTIMUM arm** (0.55/patient/10

weeks; RR 0.59 vs aripiprazole augmentation, $P=0.02$). Sequencing recommendation: start XL 150 mg ~2–3 weeks before June 8, hold ≥ 4 weeks before any uptitration given CYP2D6 interaction with duloxetine. See new [handout #21](#).

WHERE THIS LANDS THE THREE OPERATIVE QUESTIONS

- **Cymbalta → Lexapro switch.** Dad has decades of good functioning on Lexapro at 10–20 mg, which is meaningful prior-responder data. The OE evidence against the switch is real but is largely about (a) Lexapro previously failed at *high doses* in 10/2025, (b) Del Casale's SNRI-favored signal, and (c) the discontinuation/activation window overlapping with the ketamine restart. The first concern is partly addressed by capping at 5–10 mg; the third is the real load-bearing problem. **Recommended position: defensible direction with sequencing caveats — run the first 2–3 ketamine sessions on the existing duloxetine background, then revisit the switch with response data in hand. If pre-ketamine timing isn't winnable, gradual cross-titration (1+ week of overlap at 30 mg duloxetine, then 20 mg, then off) and cap Lexapro at 5–10 mg.** See [handout #06](#).
- **Add Depakote.** No defensible indication; serious drug-induced parkinsonism risk in highest-risk Arnaldi profile; would confound the U-M Movement Disorders eval. **Recommended position: defer until indication is stated; if pursued, monitoring protocol per handout #01.** See [handout #01](#).
- **Remove trazodone + doxepin; add lemborexant.** Lemborexant alone (as an addition) is reasonable and Julane-endorsed; the doxepin → lemborexant *substitution* is seamless per OE #13. Removing trazodone-PRN simultaneously serves no purpose. **Recommended position: if Dad wants to come off doxepin, the swap to lemborexant 5 mg HS is defensible night-1; keep trazodone PRN.** See [handout #10](#).
- **Alternative augmentation (new live option): bupropion XL 150 mg.** If the conversation tomorrow lands on "we want to add something beyond ketamine and beyond the doxepin/lemborexant sleep adjustment," bupropion augmentation has stronger evidence than divalproex and a more defensible safety profile than Lexapro. Cautious trial; fall risk monitoring; sequenced 2–3 weeks ahead of the June 8 restart. **Recommended position: defensible third-line (behind ketamine + duloxetine continuation + behavioral); not first-line.** See new [handout #21](#).

WHAT WE STILL DON'T KNOW

- **Whether Chilton specifically "recommended" Depakote** or simply noted it was on the chart. Dad's memory is hesitant; Mom recalls Dad saying she recommended it. No written record.
- **What other recommendations Chilton may have made** that Dad didn't note.
- **Exactly what Dr. Rubin is recommending** — Rubin call tomorrow (5/20) at 11 AM.
- **What an A2 clinic intake looks like operationally** — protocol, room arrangement, in-room provider presence, pre-session anxiolysis approach.
- **Whether Dad is open to a single pre-session benzodiazepine dose** as part of the A2 intake.
- **Whether the post-session 5/18 coherence persisted or faded** over the next 24 hours.

FOLLOW-UPS

1. **Capture call notes after each provider conversation going forward.** Mom and/or Dad after each call.
2. **Tomorrow's 11 AM Rubin call:** the Cymbalta-switch and divalproex questions are now answerable from evidence. The conversation worth having is whether bupropion augmentation makes more sense than the current proposal as an alternative form of "doing more."
3. **Email Chilton:** confirm in writing whether divalproex was recommended or noted; ask for the evidence basis behind preferring Lexapro over Cymbalta as the ketamine background, given Del Casale 2025.
4. **Get Julane's written read** of yesterday's session and her sequencing preference.
5. **Confirm A2 clinic intake details:** date (June 8), protocol, in-room provider, pre-session anxiolysis approach.

RECOMMENDED PATH

1. **Push for sequencing on the Cymbalta → Lexapro switch:** ask Rubin to run the first 2–3 ketamine sessions on the existing duloxetine background, then revisit the switch with response data in hand. Preserves the documented Feb 2026 responder background through the highest-stakes window. If pre-ketamine timing isn't winnable, accept the switch but ask for a gradual cross-titration (≥ 1 week duloxetine 30 mg overlap, then 20 mg, then off) and a Lexapro dose cap at 5–10 mg (the historical effective range, not the previously-failed high-dose range). Explicit symptoms that would trigger reversal (akathisia, persistent agitation, new motor symptom in first 2 weeks) defined up front.
2. **Defer divalproex** until an explicit indication is stated. If it proceeds despite that, full monitoring protocol per handout #01 with discontinuation triggers for any new motor or cognitive symptom.
3. **If doxepin comes off, swap to lemborexant 5 mg HS** on a single-night transition; keep trazodone 50 mg HS PRN; melatonin continues.
4. **If augmentation beyond ketamine is desired, bupropion XL 150 mg added to duloxetine is the strongest alternative to divalproex.** Start 2–3 weeks before June 8; hold at 150 mg ≥ 4 weeks before any uptitration; weekly check-ins with focused gait/balance and orthostatic monitoring.
5. **Weekly Julane sessions through the gap to June 8** — the strongest non-pharmacologic lever for the "felt alone" problem 5/18 surfaced. Use them for set/setting work, intention-setting, normalization of dissociation, and direct integration of last week's session.
6. **NPI top-tier items from [handout #19](#):** caffeine at 1 cup AM (already in place); bright-light parameters confirmed; digital CBT-I started; exercise to 3 $\times$ /week; alcohol abstinence. PSG with RBD montage prioritized in the U-M Movement Disorders coordination.
7. **Plan A2 intake with explicit pre-session anxiolysis** — single-dose lorazepam pre-session, in-room provider presence, set/setting continuity with Julane.

HOLDING ALONGSIDE THE CLINICAL *Dad has explicitly asked the family to maintain genuine hopefulness rather than false hope. Recovery from severe TRD of this duration is a long, slow climb, not a single intervention. Ketamine in the best of circumstances was the first step on a longer path, not a shortcut to the top. The 5/18 session was a hard step; it does not change the path or the destination. Continuing weekly Julane sessions through the gap is the form genuine hope takes in practice — the work is happening regardless of any infusion timing.*